1. INTRODUCTION

Hope Hospital Nagpur is committed to provide safe environment for the patients, visitors, consultants and its employees. Safety is given foremost priority in the hospital. The hospital organizes regular safety related programmes and also invites experts in the related field for guidance

The hospital has prepared its safety manual keep in mind the objective of ensuring maximum safety to all. The manual is divided into various sub sections to cover all internal and external safety related issues. The management takes keen interest in providing training to all the staff and efforts are made to make patient also aware of these issues.

This manual is reviewed at least once in a year or as and when the situation demands.

Disasters, whether natural or man made, usually occur without much warning. In the case of fire and earthquake, one may not get any warning whatsoever. Even if the probability of occurrence of such disasters may be considered as low, the necessity of being prepared for the worst has to be accorded topmost priority. Advance planning, therefore, has to be done and all the planned activities adequately rehearsed so that if the need arises, all concerned are prepared to take immediate action

2. Hospital disaster Committee:

- Dr. B K Murali – MD
- Mr Vivek Panchbuddhe– Center Head
- Mr Shrikant Bhalerao- Administrator
- Dr. Abhijit Fuladi-Medical Superintendent
- Ms Monica Ganvir- Project Manager

3. DEFINITIONS USED IN THIS PLAN

3.1. **STANDBY ORANGE ALERT**: Prepare for the possibility of an **ORANGE ALERT.** Usually precedes the alert. Dispense in-house employees to the Personnel Pool for sign in, call in on-call staff, and lock all doors except the Front Entrance and the Emergency Room doors.

3.2. **ORANGE ALERT**: Impending disaster with a count of victims being rescued and transferred to the hospital. Organize treatment and triage areas, continue with Disaster Plan assignments, initiate calling of employees, prepare to triage and treat victims.

3.3. **ALL CLEAR**: Discontinue Disaster Plan. Disaster is finished. All victims have been treated. Disaster treatment areas can be dismantled.

3.4. **MESSENGER**: Writes down messages and transports them to the intended receiver.

3.5. **TRANSPORTER**: Transports victims by carts, wheelchairs or accompanies them ambulatory and remains with the victims until released by the person in charge of the area.

3.6. **GUARD**: Stationed at doors and checks identification of persons attempting to gain entrance to the facility. Sends people without proper identification to Control Center for identification.

3.7. **INTERNAL DISASTER**: A need for extra hospital personnel to care for patients and possible evacuation of them due to an accident within the facility such as fire, tornado or explosion.

3.8. **EXTERNAL DISASTER**: A disaster which occurs outside the hospital, somewhere in the community, when there is a disproportionate amount of hospital staff to care for the incoming Emergency Room patients or victims.

3.9. **ON-CALL STAFF**: The members of the various hospital departments who are on call for hospital business on a 24 basis.

3.10. **CRITICAL**: Vital signs are unstable and not within normal limits. Patient is acutely ill or unconscious. Indicators are questionable or unfavourable.

3.11. **NONCRITICAL**: Vital signs are stable and within normal limits. Patients conscious and can be either comfortable or uncomfortable. Indicators are favourable or excellent.

4. Aim:

4.1 To provide policy for response to both internal and external disaster situations that may affect hospital staff, patients, visitors and the community.

4.2 Identify responsibilities of individuals and departments in the event of a disaster situation.

4.3 Identify Standard Operating Guidelines for emergency activities and responses.

5. Situation and Assumptions:

5.1 Several types of hazards pose a threat to the hospital:

- Internal disasters: fire, explosions, and hazardous material spills or releases.
- Minor external disasters: incidents involving a small number of casualties.
- Major external disasters: incidents involving a large number of casualties.
- Disaster threats affecting the hospital or community (large or nearby fires, impending tornado, flooding, explosions, Bomb threat etc.).
- Disasters in other communities.

5.2 Natural Disasters: We should be prepared for the following:-

- Earthquakes
- Cyclones
- Floods
- Lightning

5.3 Man-made Disasters

- Violence/terrorism resulting in mass casualties
- Accidents including train accidents involving large number of personnel
- Fire

- Food poisoning
- Epidemics

5.4 Exclusive to the Hospital

While all the disasters mentioned above affect everyone in the area of occurrence, there may be certain emergencies within the hospital which may be as follows: -

- Spillage of hazardous material such as biomedical waste etc.
- Civil disorders affecting the hospital (including problems created by antisocial elements).
- Failure of electric supply.
- Bursting of water pipelines.
- Fall of patients from beds or trolleys.
- Leakage of Aesthetic gases.
- Bomb threats

5.5 Of the above, all the types of natural disasters as well as fire deserve special attention, precautions and preparations as the hospital itself may be affected in such an eventuality. As far as the others are concerned, we will be involved in medical help in such emergencies for which necessary plans should be made. For the former, a hospital Safety Committee will be formed who will oversee and execute the necessary arrangements.

6. General Considerations:

6.1 Lines of Authority: The following persons, in the order listed, will be in charge:

1. Dr. B K Murali – MD
2. Mr Vivek Panchbuddhe– Center Head
3. Mr Shrikant Bhalerao- Administrator
4. Dr. Abhijit Fuladi-Medical Superintendent

5. Ms Monica Ganvir- Project Manager

6.2 Communications:

- A Command Centre will be set up at the Security Desk to handle and coordinate all internal communications. All department heads or their designee will report to this office and call as many of their employees as needed.
- The person in charge when the disaster happens will assign a nurse to the communications system in the E.R. This nurse will answer all calls from this station.
- At least one messenger will be assigned to Public Address System to deliver messages, obtain casualty count from triage, etc.
- Person directing personnel pool shall send a runner to all departments to advise them of the type of disaster and number of victims and extent of injuries when this information is available.
  - Nursing will be notified by the in-charge or designated persons.
  - Department Heads will notify their key personnel.
- A "Visitor Control Center" will be set up in the front lobby. Families of casualties will be instructed to wait there until notified of patient's condition. Normal visiting hours will be suspended during the disaster situation (orange alert).
  - A hospital staff member will stay with the family members.
  - A list of the visitor's names in association with the patient they are inquiring about should be kept. Housekeeping staff may be needed to escort visitors within the facility.
- Telephone lines will be made available for outgoing and incoming calls. One line will be designated as the open line to the external Command Center. The person in charge will designate assigned staff to monitor the phones.

6.3 Supplies and Equipment:

- Extra supplies will be obtained from purchasing personnel through runners.
- Outside supplies will be ordered by the administrator and brought into the hospital.

6.4 Valuables and Clothing:

- Large paper or plastic bags are available in the pharmacy for patient's clothing and valuables.
- This must be handed over to the patient relatives if present or sealed in the bag Security office.

6.5 Public Communication Center:

- A communication center for receiving outside calls and giving information to the press, and relatives shall be set up in Front office.
- The press can use the parking as their headquarters.

6.6 Morgue Facilities:

- Patients pronounced DOA will be tagged with a Disaster Tag...do not remove personal effects. The top sheet from the tag will be taken to the Command Center in Emergency Department for casualty list purposes.
- After bodies have been identified, the information will be filed on the Disaster Tag and Medical Records notified as to the identification of the patient.
- The bodies will be handed over to Police and removed .A complete record of all bodies must be maintained along with the name of the agency removing them, e.g., police, fire department etc.
- Be sure appropriate paperwork is filled out as per MLC procedure.

7. Responsibilities of Individuals and Departments:

7.1 Centre Head:

In a **major disaster** will do the following functions

- Check with local authorities to verify the disaster and obtain additional information.
- Authorize announcement of disaster to hospital personnel.
- Ask for help from local police and volunteer organizations as deemed necessary.
- Stay in the area of administrative offices to be available to assist, as requested/required.

7.2 Administrator:

- Is responsible for determining the extent of the disaster, whether it is a "major" or a "minor" disaster. If it is a major disaster,
- In a **major disaster** will do the Administrator's functions, if he is absent.
- Is responsible for notifying all department heads or alternates.
- In a **major disaster** be responsible to see that families of victims are notified as soon as possible. These calls may be made by the physician who treats the victim.

- The Command Center Director will coordinate these efforts and notify center head as to when information can be released to the press.

7.4 Front Office (IP reception)

- Assign responsible person to PA system as soon as possible.
- Department head or designee will call in their own personnel as needed after having reported to the Command Center.
- Notify Communications/ Security if internal disaster is involved.
- After notification of disaster by authorized person, you are responsible for sounding the "Orange Alert" alarm with the help of Communication cell.
- Do not accept routine non-emergency admissions.
- Refer all public information calls and press to help desk in Reception Area.
- Direct press to the parking.

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7.6 Maintenance

- Department head or designee will call in their own personnel as needed after reporting to Command Center.
- Maintain full operation of all facilities/ equipment.
- All doors should be locked/Blocked immediately **except** the main entrance.
- Be willing to help with movement of victims from ambulance to Triage.
- Be prepared to obtain additional Ventillators and equipment as needed.
- Be prepared to assist in treatment areas.
- Keep resuscitation equipment in good operating condition and well marked.

7.7 Housekeeping and Laundry

- Department head or designee will call in their own personnel as needed after reporting to Command Center.
- Be responsible for setting up extra beds in hospital if needed, as well as transporting storeroom supplies and bringing in extra supplies from other areas.
- Be available to help clean receiving area, and clean rooms between cases in treatment areas.
- Be sure all traffic areas are clear of cleaning carts, equipment and etc.

7.8 Operating Room

- Nursing In charge OT will supervise Operating Room and call all needed personnel after reporting to Command Center.
- Call additional surgeons as needed.
- Check area for supplies and equipment.
- Ask for additional help to carry out surgery and treatments in Operating Rooms and Recovery Room.
- Assign and direct scrub nurses and circulate.
- Notify Triage when Operating Rooms and Recovery Room is available for more patients.
- Keep minimum list of supplies on hand and be prepared to process additional sterile supplies quickly.
- Notify anaesthetists who will maintain adequate anaesthesia and drug supplies.

7.10 ICU - After notification of disaster, the ICU nurse will:

- Evaluate patients in the Intensive Care Unit for possible discharge. Use established discharge criteria as a guide. Transfer patients out if indicated.
- Prepare to admit more critically ill patients.
- Send runner to Command Center or phone for help.

7.14 Medical Imaging

Day Shift:

- The department head or designee will find out the number of patients involved and any other pertinent information from the Command Center.
- The department head or designee will be responsible for calling in any and all personnel needed to sufficiently handle the patient load.

Evening Shift:

- The technician on duty or on call for the Radiology Department will be alerted by the night supervisor. This technologist will be considered the designee of the x-ray department and will report to the information center for further information.
- It will be the duty of this technologist to call in extra help as needed. All extra help called in will report directly to Radiology.

Duties of Medical Imaging Personnel

- Call any or all personnel needed.
- Arrange for extra supplies to be brought in if needed.
- Coordinate flow of work and delegation of work areas. .

7.15 Laboratory

- Department Head or designee will call in their own personnel as needed after reporting to Command Center.
- Call personnel from nearby hospitals and clinics as necessary.
- Have arrangements made to obtain additional blood, equipment and supplies from out sourced agencies.

7.16 Pharmacy

- Department Head or designee will call in their own personnel as needed after reporting to Command Center.
- Be prepared to supply all departments with needed supplies.
- Director will designate assistant to supply runners to deliver supplies.
- Have an up-to-date list of suppliers who can quickly supply extra materials.
- Report to Command Center, then remain in department.
- Have list of drug suppliers that can provide emergency supplies quickly Keep minimum supply of emergency drugs on hand at all times.
- Pharmacy should remain open and have a runner to deliver needed meds to areas. .
- Keep adequate supply of bubblers, cannulas, masks and flow meters available in the Hospital.

7.19 Physiotherapy

- Be prepared to accept walking wounded victims. Be prepared to provide assistance
- Request a runner from Command Center as needed.

7.21 PRE

- Report to the Command Center and be prepared to stay with relatives of victims in hospital lobby.
- Will provide Command Center with a list of the family members that are here.

7.23 Security

- Report to Command Center.
- Assist as needed.

7.24 Nursing Personnel Assigned to Disaster Victims

- Obtain information and fill out available information and time on disaster tags. Even if no information is available as to identity, give information as to condition, types of injuries, etc.
  - Be sure to use hospital disaster tag number for identification (the tag is in triplicate).
- BE SURE top sheet of disaster tag is made available to Medical Records with pertinent information.
- **DO NOT** leaves your patient unattended. Patient may be signed off to person in charge when admitted to a unit.
- Give aggressive first aid treatment.
- Make out the appropriate x-ray requisitions with disaster number. It is essential that they have these slips made out.
- Patients who have been admitted to the hospital should have the information slips placed with the Command Center in the Emergency Department.
- If a patient is transferred, be sure to indicate on the tag to which hospital he has been sent.
- If a patient is admitted to our hospital, be sure and send all oxygen equipment to his room with him.
- Sign disaster tags.

7.25 Medical Records/ stores

- Medical records executive will call in their own personnel as needed after reporting to the Command Center.
- Assign person to be responsible for maintaining casualty lists and assist with paperwork as needed at Command Center.
- Supply extra forms as needed.

Business Development

1. Be responsible for releasing information to the press with the instructions from center head after the families of the victims have been notified.
2. Attend to the Press personnel.

7.26 Plan Development and Maintenance:

- This Disaster Plan was developed by the Safety Committee and with the cooperation of all departments in the hospital.
- All departments are responsible for maintaining an up-to-date disaster manual and notifying the Safety Committee of changes in their departments.
- This plan will be updated annually or as changes in departments occur.

8. STANDARD OPERATING GUIDELINES

8.1 Internal Disaster Plan

8.1.1 Evacuation for Internal Disasters

A. Reasons for Evacuation

- To remove patients and personnel from actual or threatened danger such as fire, explosion, enemy attack, tornado, etc.
- To free hospital beds for the care of incoming casualties.

B. Implementation of Internal Disaster Plan

- The communication cell will be informed that an "internal disaster" is in effect. If fire is involved, they will call Fire office and Security. They will immediately notify all departments.
- Patients will be evacuated from stricken areas to "safe" areas of the hospital.
- The decision, as to the extent to which the plan is to be implemented, will be determined by the person in charge.
- Roll call will be taken before, during and after evacuation, if possible.

8.1.2 Resources for Patient Transfer

- Other area hospitals can be contacted for transfer of patients, additional supplies, or additional personnel.

- Contact the Administrator , if these other hospitals need to be contacted. They will need to know the number of patients to be transferred and the diagnosis or types of injuries.
- Hospitals contacted will be asked to provide emergency ambulance pick-up for these patients if necessary or private ambulance services will be contacted.

8.1.3 Inter-Hospital Transfers

Policy: If, in the judgment of the physician or emergency physician (in conjunction with the private physician), the patient cannot be adequately cared for at this hospital; the patient should be transferred to an appropriate Government / private care facility. This applies to the following conditions:

- Where patient expresses his unaffordability to meet the expenses. In such case patient should be given preliminary treatment, stabilized and then transferred.
- Severe burn patients that require burn center.
- Condition where the specialty physician cannot be reached, i.e. pediatric emergencies, complicated orthopedics, etc.
- The specialty physician desires to transfer the patient after stabilization and examination.

8.1.4 Procedure for Transfer:

- A copy of the patient's chart or treatment record accompanies him with all charting completed. X-rays and lab reports will be sent at the discretion of the transferring doctor.
- That facility shall be contacted prior to the transfer and agree to accept the patient.
- Physician to physician or CMO to CMO contact by telephone should take place prior to transfer, whenever possible.
- A transfer summary should be completed and signed by the transferring physician, the patient, if possible.
- The patient has been adequately prepared for transfer according to his condition (IV established, pressure dressings, immobilization of fractures, patient airway) and is accompanied in the ambulance by competent personnel.
- A nurse to nurse contact should also take place. This can be done after patient has left for transfer.

8.1.5 Hospital Evacuation Plan

- In the event of fire or other internal disaster, all patients and personnel will have to be removed from immediate danger to a safer section of the building, or removed from the building.
- Moving will be done first to other wing on the same floor and then if those areas become dangerous patients and personnel will be moved to lower floors or to the outside of the building.
- Moving will be done in a systematic fashion by moving all patients and personnel closest to the danger first.

Remember keep cool, calm, and collected and we should have a successful evacuation.

If we evacuate the entire building, everyone will report to the parking lot. A roll call will be made of each department area involved by the person in charge of each area. The senior front office assistant will check with each department head to make sure everyone is accounted for.

8.2 External Disaster Plan

General Directions for the Implementation of the External Disaster Plan

8.2.1 Day Shift

- The nurse receiving the communications Communication cell or the office should implement the first step of the disaster plan by notifying the HA office that a disaster situation exists and requesting them to notify the following:

a. MD / Center head/ Administrator

- The nurse in immediate charge will notify the doctor (or doctors) on call or she will ask another nurse to notify them.
- PRE/HK supervisor, at the direction of the administrator will then make the following announcement on the PA System and repeat it 3 times: "YOUR ATTENTION, PLEASE...ORANGE ALERT."
    - At this announcement, all department heads will report to the Command Center at ER desk for instructions.
- Disaster manuals will be kept available by all department heads.
- All patients involved will be triaged through the Receiving Station (the casualty).
- The "Command Center" will be at the managing director’s chamber
- Additional nurses should be requested through the Command Center. Other department heads or their alternates will contact the number of additional personnel they need.

8.2.2 Evening (After 5.00PM) and Night Shifts and Holidays

- The night supervisor will implement the first step of the disaster plan by announcing the Orange Alert and notifying the MD/Center head/ Administrator
- She will then call the nurse on duty in the hospital unit who will notify the following people:

a. CMO/casualty Nurse, who will assume triage nurse duties.

b. Doctor on call...then all other doctors on staff, as deemed necessary.

c. Other "on call" people (e.g., surgery, lab, x-ray, etc).

- Department heads at home will be called by communications on the instructions of MD/Center head/ Administrator.
- The Night supervisor will be in administrative charge until a person higher in line of authority arrives.
- The on-call physician will be in RMO incharge.

8.3 Expansion of Facilities

8.3.1 Reception of Patient

- A receiving and sorting station will be established in the casualty where all patients will be quickly tagged with a Disaster Tag and will be known by the number on the tag until more information is available.
- Tie hospital disaster tag to ambulance tag with hospital tag on top. Put admitting time on tag. Send to appropriate Treatment Areas for lifesaving care with nurse assigned to each patient.

8.3.2 Treatment Areas

- OT - Immediate surgery required.
- Outpatient area a – Walking wounded waiting for care.
- Holding area for surgery and transfer patients.
- Casualty - Patients needing life-support care.
- ICU Care or direct hospital admits - Major medical patients.
- Basement -Body holding

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8.3.3 Personnel Assignments

- Specific assignments will be issued to personnel by their department heads. Department heads should have more than one alternate.
- All employees reporting on duty will go to their department. Their department head or alternate will then assign specific duties. (All nurses, nursing assistants and executives report to Command Centre.)
- The nurse assigned to a patient being moved from Triage must stay with this patient until relieved by another competent person or until the patient is admitted to a unit and signed off to appropriate person in charge of that unit.

8.3.4 Records

- Records should be minimized, depending on circumstances.
- Numbered disaster tags will be available in the Receiving Area and affixed to EVERY patient until the proper outpatient chart or hospital chart is prepared. Sufficient information should be recorded in order to aid in identification and determination of extent of injuries. This should be done in Treatment Areas rather than Receiving.
- Report any major change of the patient's condition to this Command Centre. Lab, and x-ray should include the patient's disaster tag number and the patient's location in the hospital.

8.3.5 Traffic Control

- If necessary, the administrator/PRO/ Marketing manager will notify Traffic police and request assistance in handling external traffic.
- Discharged hospital patients will leave by the Wardha road gate. The admitting office will station personnel there to discharge patients.
- Elevators will be utilized only for the movement of patients or equipment or medicine supply.
- The transfer of victims to other hospitals will take place at the Emergency Department entrance. The Holding Area for the transfer patients will be the Waiting Area on ground floor until the Emergency Department is cleared and cleaned.

8.3.6 When Additional Bed Space is Needed

- Empty beds in the hospital will be utilized first.
- If necessary, the medical staff will discharge the following types of patients until adequate space is available:

a. Diagnostic problems and observation cases that are not bedridden.

b. Patients about to be discharged.

- Discharged hospital patients will leave by the back gate to avoid incoming casualties. Contact Front Office for assistance.
- The PRE will call relatives of these patients for transportation.
- Routine admissions will be discontinued until it is determined that space is available.

Extra cots, mattresses, and beds in storage will be set up in vacant areas by housekeeping staff under the guidance of administrator As a temporary measure, patients can be placed on blankets or mattresses on the floor.

Extra cots will be obtained if necessary from other area hospitals.

8.4 Severe Thunderstorm/Tornado Protocol

If a tornado or unusually severe storm appears inevitable, precautions should be taken that include the following:

- Draw all shades and close all drapes as protection against shattering glass.
- Lower all patient beds to the lowest position, and move the bed away from the windows as much as possible.
- Place blankets on all bed patients.
- Close all doors.
- Get as many ambulatory patients as possible into the basement.
- Be prepared to sound the Orange Alert.
- Do not use the elevator.

8.6 Bomb Threat Procedure

8.6.1 Receipt of Warnings:

1. When a phone call is received:
    - Prolong the conversation as long as possible;
    - Be alert for distinguishing background noises, such as music, voices, aircraft, and church bells;
    - Note distinguishing voice characteristics;
    - Ask where the bomb will explode, and at what time;
    - Note if the caller indicates knowledge of the hospital by his/her description of the location.

2. Notify authorities and key personnel:
    - POLICE
    - MD/Centre Head/ Night supervisor .

8.6.2 Search Procedure:

- After the basic details are provided by the person receiving the call, the Administrator or his designee should make all the necessary decisions, issue orders, and prepare for the arrival of assistance.
    - Police should be put in complete authority upon arrival. Cooperation with the police and others involved is most important. Hospital personnel with master keys should be available.
- The Administrator must depend upon his key personnel and the equipment immediately available.
    - Local authorities may not be as familiar with the floor plan as hospital personnel, nor will they have sufficient manpower to conduct an adequate search within a reasonable period of time.
    - Therefore, the building will be divided into sections, and certain employees be made responsible for the search in each assigned area.
- Watch for and isolate suspicious objects such as packages and boxes.
- Public areas such as the lobbies, canteen, public toilets, and stairwells should be thoroughly searched.
- The search should be thorough, eliminating those areas which are locked and unavailable to the public.
    - If the caller indicates the area in which the bomb is located, this area should receive immediate attention. Tight security should be maintained on each area searched until the entire search is completed.
- Elevators should be kept available for local authorities.
- If what appears to be a bomb is found, DO NOT TOUCH IT. Clear the area and obtain professional assistance. Also, try to isolate the object as much as possible by closing doors.
- Generally, personnel should remain calm and alert. Personnel should be properly trained so that patients will not become alarmed.
    - Notify the Administrator or his designee promptly of significant developments, and do not divulge to the patients that a bomb threat has been received.

- In the event the patients do learn what is taking place, they should be reassured that all is well.

8.6.3 Evacuation:

If a bomb is found, the police will notify the proper authorities to come and disarm it. We WILL NOT EVACUATE unless a bomb is found. If evacuation becomes necessary, this will be a decision of the Administrator or person in charge and the police.

8.6.4 Reports:

Each person involved should report to the Center Head /Administrator immediately after a thorough search of his area has been completed indicating the results of the search.

Key personnel should prepare written comprehensive reports to the Administrator outlining any difficulties encountered during the incident. These reports will be used to update or revise the existing bomb threat procedure.

8.7 Hazardous Material Incidents Protocol

- Protect yourself.
- **Approach** the scene with caution.
- Attempt to identify the hazardous material.
- Obtain further information and assistance for equipment, etc.
- Avoid contact with hazardous material and person.
- TRY to contain the material as much as possible in one place. Try decontaminating the person, as much as possible at the scene. Setting up the decontamination area should be done ASAP.
- With few exceptions WATER is the UNIVERSAL ANTIDOTE. For biological hazardous materials use BLEACH.

8.7.1 General Management of Decontamination Following Radiation Exposure

Before undertaking decontamination, detailed monitoring should be done and recorded. If clothing is contaminated, remove it carefully and slowly so that deposited material does not become airborne. Wear gloves, gown, mask, and shoe covers to protect yourself from contamination.

Place all contaminated material into large plastic bags and seal them. Ensure that no unauthorized personnel are allowed into the patient decontamination area. In practically

all contamination cases, the material is easily removed by a gentle soap and water washing.

After the clothing has been removed, it is usually the face and hands that remain as contaminated areas. During the general survey of the patient any abrasion or break in the skin integrity should be carefully searched for.

Any such areas should be covered by a water proof adhesive dressing to prevent contamination and subsequent internal incorporation of the contaminated material. In washing a contaminated area, wash from the periphery towards the center of the most contaminated area.

Use paper towels to mop up the contaminated soapy water. It is imperative to treat the skin gently. Do not use an abrasive scrub or strong detergent. Do not shave hairy areas. If redness or tenderness of the skin develops the procedure should be stopped.

It is important to realize that complete decontamination of the skin is not something that has to be achieved at all cost within the shortest possible time.

If externally contaminated skin cannot be cleaned after several tries, then a skin cream is applied and time is allowed for the skin to heal before repeated attempts at cleaning the following day.

General treatment for internal contamination is an involved process that is specific for specific isotopes. For example, for radioactive iodine the patient is given large doses of non-radioactive iodine to prevent uptake of the radioisotope by the thyroid gland.

Certain radioisotopes such as Tritium can be flushed from the body by volume diuresis.

Consider transfer to Decontamination Center.

Acute Management of Chemical Burns

Acute management of chemical burns, as with all emergencies, must begin with assessment of the patient's airway, breathing and circulation.

The next step is to effectively remove the offending chemical. The patient should be completely undressed and all identifiable particulate matter removed. Pooling of the chemical in skin folds, nail folds and hair bearing areas should be identified.

Immediate and prolonged irrigation should be sought. Immediate and prolonged irrigation should then begin. Care should be taken to protect personnel from the chemical. Successful irrigation may be monitored by checking the pH of the wound.

Once emergency therapy has been started, a specific history of the injury should be taken. This ideally includes the name and concentration of the chemical, duration of exposure and general health of the patient. Adequate tetanus prophylaxis is always necessary.

Repeat examinations of the burn wound should be made after several hours, at twenty-four hours and at one week.

Hospitalization of patients with chemical burns should be considered for those with debilitating illness, involvement of the eye, face, hand, foot, and perineum, burns greater than 15% TBSA and deep burns.

8.8 Release of Information to the News Media

The objectives of this guideline are:

1. To protect the privacy, health, and welfare of patients.
2. To address the public's need for information.
3. To improve the flow of information and avoid conflicts which may arise between the news media, hospitals, and physicians.

8.8.1 Hospital Spokespersons:

The Center Head is authorized to release information to the media based on these guidelines. If he is unavailable, he may delegate this to administrator, manager or a qualified staff member who has knowledge of the situation and is familiar with these guidelines.

In the case of a disaster, a communication center will be set up. MD, Center head are authorized to release information to the media.

When information cannot be released, the release should be refused with an explanation. If delays are encountered, the media should be so advised.

8.8.2 Release of Physician's Name:

The hospital may give news media representatives the name of the attending physician **only with the physician's consent**.

If that consent is given, the hospital may refer representatives of the media to the attending physician for information relative to the case.

8.8.3 Privacy Rights of Patients

- Confidentiality (non-emergency patients) - Patient health care records or information regarding patients may be released only with the informed written consent of the patient or of a person authorized by the patient.
- This same form should be used for "newsworthy persons" such as public officials or others who are prominent or well known in the community.
- Emergency Situations - When consent is not immediately available.
- Where the identity is known by the media from other sources such as law enforcement personnel, basic information as listed under Patient Condition and Information Categories can be released.
- Where patient identity is unknown, general information on patient conditions without releasing identities is allowed. It's very important not to release the identity of accident victims before family members have been notified.

8.8.4 Patient Condition

- Treated and Released
- Refused Treatment
- Refused Hospitalization
- Good: Vital signs including pulse, respiration rate, temperature and blood pressure are within normal limits. The patient is conscious and comfortable.
- Fair: Vital signs are within normal limits, but the patient may be uncomfortable or suffering minor complications.
- Serious: Vital signs may be outside normal limits and treatments are being directed toward assisting return of some vital signs to normal limits. There may be signs of organ system instability and mechanical systems may be necessary.
- Critical: Vital signs are frequently outside normal limits and one or more organ systems are in failure. There are major complications responsive to outside stimulation or in a coma.

8.8.5 Information Categories

- Blood Loss: Slight, moderate or severe loss of blood may be reported. If transfusions are required, this may also be reported. The amount of blood transfused or blood lost in moderate or severe cases should be given if known.
- Burns: State the extent of the burns, the specific part of the body burned and percentage of total body burned (40%, 50%, 60%, etc.). The source of the burn, e.g., steam, sun, fire, chemical, may be given.
    - First degree - redness
    - Second degree - blistering and swelling of skin
    - Third degree - total destruction of the skin

- Foreign objects: Foreign objects removed from a patient may be identified.
- Fractures: Indicate the part of the body involved, e.g., arm, leg, wrist, foot, rib, and whether simple (bone not protruding from skin) or compound (bone protruding from skin).
- Frostbite: State the extent of the frostbite and the specific part of the body frostbitten.
    - First degree - redness
    - Second degree - blistering and swelling of skin
    - Third degree - total destruction of the skin
- Head Injuries: State that the head is injured. If a skull fracture is confirmed by x-ray, that can be reported.
- Internal injuries: It can be stated that there are internal injuries.
- Lacerations (cuts): Report the general area of the body that was cut, e.g. arm, leg, chest, whether there are single or multiple lacerations, and whether any or all of the lacerations required stitches,
- Poisoning: Cases of suspected poisoning may be reported. The kind of substance ingested, if known, can be reported - such as weed killer, detergent, house cleaner, medicine, etc.
- Shooting or stabbing: It is permissible to state the wound was a result of a shooting or stabbing. The wound may be described as penetrating or grazing. The general part of the body wounded should be described.
    - The exact type of weapon used (e.g. butcher knife, 32 caliber revolver) should not be released.
- Unconsciousness: If a patient is unconscious when brought to the hospital, a statement of that fact may be made.

8.8.6 Information to Medico-legal Consultant

Requests for details other than the routine information given on any patient must be referred to the Medico-legal Consultant on any of the following:

Any death wherein the body is unidentified or unclaimed.

All sudden deaths not caused by readily recognized disease, or wherein the cause of death cannot be properly certified by a physician on the basis of prior (recent) medical attendance.

All deaths occurring under suspicious circumstances, including those where alcohol , drugs, or other toxic substances may have direct bearing on the outcome.

- All deaths occurring as a result of violence or trauma, whether apparently homicidal, suicidal, or accidental (including those due to mechanical, thermal, chemical, electrical, or radiation injury, drowning, cave-ins) and regardless of the time elapsing between time of injury and time of death.
- All fetal deaths, stillbirths, or death of any baby within 24 hours after its birth, where the mother has not been under the care of physician.

8.8.7 Confidential cases where no information should be released

In a number of situations, no information - absolutely none - should be released about a person's treatment or hospitalization. Such confidential cases specifically include the following:

Persons admitted for mental illness

Persons admitted for drug or alcohol abuse

Births to single parents

Suspected or known suicide attempts

Victims or rape or other sexual assault

Suspected abuse of children

Cases of sexually transmitted disease

Persons who have specifically requested confidentiality

In response to inquiries regarding patients who fall into one of the above categories, the hospital spokesperson should simply indicate that the case falls into a category for which public comment is prohibited, and therefore no information on the matter may be released.

This also means that no acknowledgment of whether the individual is receiving care at the hospital may be made.